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Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy
Department of Health and Human Services
Mary E. Switzer Building
330 C Street SW, Washington, DC 20201

RE: RIN 0955-AA13; Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Dr. Keane:

On behalf of Providence, thank you for the opportunity to provide feedback in response to the Request for Information (RFI) on Accelerating the Adoption and Use of Artificial Intelligence (AI) as Part of Clinical Care that was published in the *Federal Register* on December 23, 2025.

Providence is a non-profit health system in the United States with a seven-state footprint that spans Alaska, California, Montana, New Mexico, Oregon, and Texas. Our diverse family of organizations employs 117,000 people who serve in 51 hospitals, more than 1,000 clinics, a health plan with more than 600,000 beneficiaries, senior services, home health, hospice, PACE, housing, and many other health and educational services. As a mission-driven organization, we are committed to understanding and responding to the needs of the many communities we serve and to providing high-quality health care for all. We have a special focus on serving hard working Americans who, despite their personal drive and commitment to provide for their families, must depend on Medicaid coverage for access to care. Each year we work to provide care and services where they are needed most, including investments in community benefit that totaled $1.9 billion in 2024. Together, we are transforming care with a holistic and deeply compassionate approach to medicine.

In this letter, we offer feedback on six areas of the RFI:

- Gaps in Regulation
- Accreditation to Promote Innovation
- Incentives for EHR Vendors
- Research and Development
- Health Care Organization Adoption
- Patient Expectations

Gaps in Regulation

Providence appreciates HHS's goal to establish a regulatory scheme for AI use that is well understood, predictable, and proportionate to any risks presented. Clear compliance expectations will help spur innovation while protecting patients and their information. From our experience developing and deploying AI-powered tools, we have identified gaps in regulation that have prevented clinical AI adoption.

Most importantly, patients need federal regulations to set expectations about consent for the use of AI in clinical care. The patchwork of state-level consent rules has paved an unreasonable pathway for both patients and providers to be confident in when consent is needed and how to appropriately secure it. For instance, can patients place restrictions on an AI tool accessing their personal information in the way they can request restrictions on the use and disclosure of their protected health information (PHI) under existing federal rules? How does the HIPAA privacy rule interact with the use of AI on PHI when the specific AI tool is owned by the provider, or a business associate, or another third party? Current regulations on patient privacy and consent have not been able to keep pace with the rapid development of AI tools in health care or the growing expectations of patients to know exactly when and how their PHI will interact with an AI-powered tool.

Similarly, providers and AI developers need federal regulations to outline accountability standards. When all entities take the required steps to vet new tools, protect patient rights, and appropriately apply AI in the clinical setting, who should be responsible when the tool fails and a never-event occurs? Federal accreditation or certification of AI tools would help all parties better understand their role in the vetting process, but even the best technology can fail. Providers need direction and support from federal regulators about where accountability should lie as we endeavor to negotiate with national electronic health record (EHR) vendors and AI companies who have greater negotiating power than even the most sophisticated health systems. HHS regulations in this area will help all parties keep the patient at the center of those negotiations.

Providence greatly appreciates the president's *Ensuring a National Policy Framework for Artificial Intelligence* executive order aimed at preventing 50 discordant state rules on the use of AI. As a multi-state health system, we believe the federal government is in the best position to dictate AI use standards that benefit patients and providers while supporting rapid innovation. At the same time, HHS should describe which areas of regulation *can* fall within a state's purview to make it clear what role if any the states should play in health care AI. Over the past year, many states have taken legislation action to regulate the use of AI in health care sensing that the federal government intended this issue to be a state one. It is important for HHS to clearly outline the states' roles and responsibilities in regulation and enforcement so that provider and developer expectations are clear and hospital resources are not futilely spent on compliance with state legislative schemes that will eventually be preempted by federal rules.

Likewise, regulations should outline the areas in which providers will maintain discretion, for instance to reject AI tools, apply more stringent vetting standards, or provide patient education and options about their care.

AI in Non-Medical Devices

One specific gap in regulation concerns the use of AI technology in non-medical devices. The FDA has published guidance for digital health innovators that seek to use AI such as large language models and machine learning in medical devices classified under the software as a medical device definition. However, there is still a gap in how the federal regulatory landscape, including FDA and HHS reimbursement like Medicare and Medicaid, will treat AI tools used in non-medical devices. We believe this is preventing developers from bringing more of these tools to market.

Recently on January 6, the FDA announced that it will ease regulation of non-medical digital health devices that do not make medical claims about diagnosis or treatment. Providence certainly shares the FDA Commissioner's goal to move regulations "at Silicon Valley speed," but this lack of regulatory standards is a barrier to clinical adoption. As described below, health care providers are wary of adopting AI-enabled devices into clinical practice without fully vetting the technology and its risks, but with federal regulatory parameters including approval from the FDA providers will be able to smooth their internal vetting process to get the technology to patients sooner.

Accreditation to Promote Innovation

HHS asked for input on how it can support private sector accreditation or credentialing to promote innovative and effective AI use in clinical care. Providence fully supports a nationally recognized accreditation path for AI tools. Absent accreditation by a third party, health care providers must invest their own time and funds to determine whether each AI tool is safe to adopt. This not only drains resources that hospitals could direct to patient care and workforce development, but it certainly slows the adoption of AI in clinical care.

Providence is proud of the standardized risk stratification process that we have developed to examine and improve AI tools before we integrate them into clinical practice. Using this process, Providence staff plots the tool's placement on a spectrum from research to direct patient care, along which patient fear and outcome risk grows as the tool moves closer to the bedside. Our system also adjusts for patient complexity. This carefully crafted stratification has informed our deployment of AI tools so that executives and clinicians are informed about the relative risk of new tools and Providence can keep pace with private innovations. But a national accreditation program would ensure all health care providers have the option to move at the speed of innovation and all patients can be informed about the rigors these accredited tools have passed.

To promote private sector accreditation or similar vetting, HHS should establish a forum for public-private partnerships to take on this work. The financial and administrative burden of a new accreditation program should not fall to the federal government, but a public-private relationship would help strengthen the profile of the accreditation and lend trust for providers and patients. By requiring accreditation for any applicable AI tools deployed for clinical care, HHS would accelerate the process, alleviate the financial burden on providers who are vetting on their own today, and remove some uncertainty about risk by knowing which parameters the accredited tools have already met.

Given the speed at which AI technology changes and improves, it will be important for any accreditation process to include frequent auditing to maintain compliance. To reduce the burden on the accrediting bodies and developers, this ongoing monitoring could take the form of a public registry of outcomes, similar to the procedure that applies to clinical researchers undergoing clinical trials, to build public confidence in the use of AI. Vendors and providers need to make sure patients continue to be confident and be in safe hands with clinical AI tools.

Incentives for Electronic Health Record (EHR) Vendors

There is little doubt that the EHR industry currently stands as a near monopoly with limited competition among a handful of large companies offering core EHR software for health care providers across the country. Within that market dynamic, it will be important for HHS to take steps to promote competition from third party AI software developers so that innovation is not stalled and patients have access to the latest and greatest improvements. That involves two key measures: 1) promoting interoperability of third-party software into the core EHR products currently occupying the market; and 2) opening the market to all interested developers to keep prices down.

Providence encourages EHR vendors to provide certain AI functionality to their customers with the company's baseline product. Vendors should not be permitted to keep AI away from providers who are unable to afford add-on after add-on.

We recommend the ASTP clearly outline when and how its certified EHR vendors are permitted to market or describe provider and patient tools as "AI-enabled" or "powered by AI." In the realm of medicine, AI has been a part of daily practice for many years – i.e., diagnostic aides, speech-to-text notetaking - but patients today understand the presence of the "AI-enabled" stamp very differently than decades ago. As the federal government proceeds with standard patient consent and/or accreditation requirements for AI technology in clinical care, it's important to make sure these requirements only apply when it is truly necessary for patient protection. For example, if Providence were to encounter a new software add-on option that the developer described as "powered by AI," it would trigger our internal risk stratification review before we could agree to adopt the tool and integrate it into our practice. It is important in that circumstance for the EHR vendor to honestly describe the use of AI in development of that tool before resources are spent to analyze the tool and its deployment is stalled when it truly doesn't use the kind of AI that required such a robust review.

Research and Development

HHS is interested in suggestions on accelerating care delivery research and implementation science, as well as AI entrepreneurship in health care. Providence recommends HHS focus its power by prioritizing research on the use of genetic data in AI tools and how to appropriately de-identify PHI as AI tools are developed to protect patient privacy.

Health Care Organization Adoption

In our experience, the adoption of clinical AI tools by health care organizations and specifically hospitals is influenced by three factors: 1) the broad network of third party providers that are involved in the hospital's ecosystem; 2) workforce concerns about job losses and new risks; and 3) lack of funding support for new technology infrastructure needed to support AI-powered tools.

In the best sense possible, health care is a patchwork of people and organizations working together to follow a patient and integrate their care needs. This makes it difficult for all providers in that patient's network to understand the technological sophistication and compliance of the others. This uncertainty has prevented the use of AI in some instances, because Providence clinicians cannot be sure whether the patient's medical record contains any AI-generated content and if so, whether that has been reviewed by a live clinician before it was sent to our hospital. We cannot be sure whether a third-party clinician used an AI tool to match a patient's symptoms to a diagnosis and what level of consent the patient has given about the use of that tool. Once the patient and their record travels more than two or three degrees beyond our health system, we simply cannot know whether AI was used or the level of risk it carries. Until all providers are operating under the same standard regulations from HHS or the tools are nationally accredited, health care organizations will be wary to fully adopt AI across the care continuum.

Health care organizations are not immune from the public's gut reaction that AI tools will take away human jobs, even in an industry that is as human centered as health. Organizational leaders are faced with opposition from union voices about the future integration of AI tools in clinical practice. Even ambient listening tools that use AI to make clinical notes following a patient visit and cut down on the clinical charting needed can spark resistance from practitioner unions who are worried about the resulting staff downsizing or increased visit frequency expectations. It is difficult to imagine how HHS can help alleviate those fears in the workforce since they are shared among all industries where AI can encroach on human jobs, but it's important for HHS to understand that this is a barrier for health care organizations moving forward with AI.

The third barrier is a lack of funding support for AI tools, and this goes beyond the funding needed to purchase the actual tools EHR vendors and third parties are offering. AI-powered technology requires more powerful wireless internet and Bluetooth capabilities. At a basic level, that means making space in the IT closet or adding new rooms to house additional wiring and fans. At a broader level, it means putting funding aside for staff training and compliance monitoring. HHS and its Centers for Medicare and Medicaid Services (CMS) can work together to ensure funding support for AI adoption by offering one-time grants for qualifying hospitals (i.e., rural and safety net hospitals) to make the IT upgrades needed for AI integration in their facilities and by ensuring Medicare reimbursement policies can keep pace with clinical acceleration that is expected as AI adoption grows. That is, clinicians would have the freedom to focus on the patient during their check-up rather than making notes on their computer and presumably those visits get shorter over time. Medicare coding and reimbursement policies need to be mindful of the ongoing costs of supporting AI technologies that are making those visits more efficient and causing the clinician to bill for a shorter time interval code with a typically lower payment rate. CMS will need to examine how to make those payment rates sustainable for AI-powered clinical practice.

Patient Expectations

As President Trump described it in his executive order in December, AI is both a technological revolution and a race. It is a topic of everyday conversation with patients around the country and their focus in our experience has been singular: tell me when you are using AI with my information. The barrier we have in meeting this expectation is the widely varied understanding on what "AI" means to patients. AI technology is so prevalent in our lives, so what do patients really want to or need to know about when it comes to their clinical care? Every email that uses autocorrect is using AI, but surely patients are not interested in giving consent to their physician using spell check tools. However, if a clinician were relying on a machine learning diagnostic tool to flag a potential cancerous lesion on an MRI image, the patient would want to understand how the clinician used that AI tool and what steps he might have taken to confirm the tool's result. Providers and patients need direction from HHS about when to disclose AI and how much disclosure is necessary and reasonable.

This is where standardized notice and consent templates from HHS would help. Providence would appreciate an opportunity to work with HHS to develop simple patient-facing language to educate the public on how AI might be used for their clinical care. For example, a hospital could deliver notice to patients in the emergency room to state “Your care team might use technology powered by AI that will help them reach a diagnosis.” If national accreditation can be established, patient consent could involve a statement such as “clinicians in our facility use AI technology certified by a federal agency, do you consent to the use of that technology?” This standardization, which was similarly used for patient opt-in to health information exchange, when provided by HHS with small customization options can go a long way to assuage patient fears about AI technology and meet their expectation to be fully informed about the presence of AI in their lives.

Conclusion

Providence is energized by the federal government’s interest in accelerating AI adoption in clinical care and we look forward to sharing our experience and best practices with HHS and other providers as this effort continues.

Thank you for the opportunity to provide feedback in response to the RFI. We hope you find our input informative.

Sincerely,

Ali Santore

Ali Santore
Chief Communications & External Affairs Officer
Providence